

ANWU E.C. TABLETS 50mg

Fluvoxamine maleate 50mg

Description

It is round, pale yellow, enteric-coated, printed "387" on one side and "ST" on the other side.

Composition

Each tablet contains Fluvoxamine maleate 50 mg.

Mode of action

Fluvoxamine is a selective inhibitor of serotonin reuptake in brain neurons. There is minimum interference with noradrenergic processes. Receptor binding studies have demonstrated that fluvoxamine has negligible binding capacity to α -adrenergic, β -adrenergic, dopaminergic; histaminergic; muscarine cholinergic, and serotonergic receptors.

Summary of pharmacodynamics and pharmacokinetics

Fluvoxamine is completely absorbed following oral administration. Maximum plasma concentrations occur within 3-8 hours of dosing. The mean absolute bioavailability is 53% due to first-pass metabolism. The bioavailability is not influenced by concomitant food intake.

Fluvoxamine is distributed extensively into human tissues, with a volume of distribution of 25 l/kg. In-vitro plasma protein binding of fluvoxamine is 80%.

Fluvoxamine undergoes extensive metabolism in the liver. Although CYP2D6 is in vitro the main isoenzyme involved in fluvoxamine metabolism, plasma concentrations in poor metabolizers for CYP2D6 are not much higher than those in extensive metabolizers. The mean plasma half-life is approximately 13-15 hours after a single dose and slightly longer (17-22 hours) during repeated dosing, when steady-state plasma levels are usually achieved within 10-14 days. Fluvoxamine undergoes extensive hepatic transformation, mainly via oxidative demethylation, into at least nine metabolites, which are excreted by the kidneys. The two major metabolites showed negligible pharmacological activity. The other metabolites are not expected to be pharmacologically active. Fluvoxamine is a potent inhibitor of CYP1A2 and a moderate inhibitor of CYP2C and CYP3A4, with only marginal inhibitory effects on CYP2D6. Fluvoxamine displays linear single-dose pharmacokinetics. Steady-state concentrations are higher than calculated from single-dose data and are disproportionably higher at higher daily doses.

Special patients groups:

The pharmacokinetics of fluvoxamine is similar in healthy adults, elderly patients, and patients with renal insufficiency. The metabolism of fluvoxamine is impaired in patients with liver disease.

Steady-state plasma concentrations of fluvoxamine were twice as high in children (aged 6-11) as in adolescents (aged 12-17). Plasma concentrations in adolescents are similar to those in adults.

Indications

- a) Treatment of depressive illness and the symptoms of depressive disorder.
- b) Treatment of symptoms of obsessive compulsive disorder (OCD).

Adverse reactions/side effects

Observed symptom associated with fluvoxamine treatment. This side effect usually diminishes within the first two weeks of treatment. Other adverse events, observed in clinical studies at frequencies listed below, are often associated with the illness and are not necessarily related to treatment.

Common:

Body: Asthenia, headache, malaise

Cardiovascular: Palpitations/tachycardia

Digestive system: Abdominal pain, anorexia, constipation, diarrhoea, dry mouth, dyspepsia

Nervous system: Agitation, anxiety, dizziness, insomnia, nervousness, somnolence, tremor.
Skin: Sweating

Uncommon:

Cardiovascular: (Postural) hypotension

Musculoskeletal: Arthralgia, myalgia

Nervous System: Ataxia, confusion, extrapyramidal symptoms, hallucinations

Urogenital: Abnormal (delayed) ejaculation

Skin: Cutaneous hypersensitivity reactions (incl. rash, pruritus, angioedema).

Rare (Frequency <0.1%):

Digestive system: Liver function abnormality

Nervous system: Convulsions, mania

Urogenital: Galactorrhoea

Skin: Photosensitivity

Psychomotor restlessness/akathisia

Other adverse events :

Weight gain and loss have been reported.

Rarely, serotonin syndrome, neuroleptic malignant syndrome-like events, hyponatremia and SIADH. It is possible that withdrawal reactions may occur on stopping therapy with fluvoxamine although the available preclinical and clinical evidence does not suggest that this treatment causes dependence. The following symptoms have been reported in association with withdrawal of the product: dizziness, paresthesia, headache, nausea and anxiety. The majority of the withdrawal reactions are mild and self-limiting.

When stopping, a gradual dose reduction may be considered.

Haemorrhage:

Very rarely, paresthesia, anorgasmy and taste perversion.'

Warning/Precautions

Warning

The possibility of a suicide attempt is inherent in patients suffering from depressive illness and may persist until significant remission occurs.

Patients suffering from hepatic or renal insufficiency should start on a low dose and be carefully monitored. Rarely, treatment with Luvox has been associated with an increase in hepatic enzymes, mostly accompanied by clinical symptoms. In these cases, treatment should be discontinued.

Precaution

Although in animal studies, fluvoxamine has no proconvulsive properties, caution is recommended when fluvoxamine is administered to patients with a history of convulsive disorders. Treatment with fluvoxamine should be discontinued if seizures occur.

Use in children:

Due to lack of clinical experience, the use of fluvoxamine in children cannot be recommended.

Use in the elderly:

Data in elderly subjects give no indication of clinically significant differences in normal daily dosages compared to younger subjects. However, upward dose titration should be done slower in the elderly, and dosing in the elderly should always be done with caution.

Fluvoxamine may cause an insignificant decrease in heartbeat (2-6 beats/min).

Contraindications

Co-administration of thioridazine, terfenadine, astemizole, cisapride, pimozide, alosetron or tizanidine with fluvoxamine maleate is contraindicated. Fluvoxamine maleate tablets are contraindicated in patients with a history of hypersensitivity to fluvoxamine maleate.

Dosage and directions for use

Anwu E.C. Tablets 50mg is to be taken orally.

Depression

Adults: The recommended dose is 100 mg daily. Patients should start on 50 or 100 mg, given as a single dose in the evening. Dosage should be reviewed and adjusted if necessary within 3 to 4 weeks of initiation of therapy and thereafter as judged clinically appropriate. Although there may be an increased potential for undesirable effects at higher doses, if after some weeks on the recommended dose insufficient response is seen some patients may benefit from having their dose increased gradually up to a maximum of 300 mg a day. Doses up to 150 mg can be given as a single dose, preferably in the evening. It is advisable that a total daily dose of more than 150 mg is given in 2 or 3 divided doses. Dosage adjustments should be made carefully on an individual patient basis, to maintain the patients at the lowest effective dose.

Patients with depression should be treated for a sufficient period of at least 6 months to ensure that they are free from symptoms.

Fluvoxamine maleate tablets at a fixed single daily dose of 100 mg is the recommended dose for the prevention of recurrence of depression.

Dosage for Paediatric Population (children and adolescents)

In children over 8 years and adolescents there is limited data on a dose of up to 100 mg bid for 10 weeks. The starting dose is 25 mg per day.

Physicians should consider age and gender differences when dosing pediatric patient. The maximum dose in children up to age 11 should not exceed 200 mg/day.

The dose should be increased in 25mg increments every 4 to 7 days, as tolerated, until maximum therapeutic benefit is achieved. It is advisable that a total daily dose of more than 50mg should be given in two divided doses. If the two divided doses are not equal, the larger does should be given at bedtime.

Obsessive compulsive disorder

The recommended starting dose is 50 mg per day for 3 - 4 days. The effective dosage usually lies between 100 mg and 300 mg per day. The dosage should be increased gradually until the effective dosage is achieved, with a maximum of 300 mg per day for adults and 200 mg per day for children from 8 years on/adolescents.

Doses up to 150 mg can be given as a single dose, preferably in the evening. It is advisable that a total daily dose of more than 150 mg is given in 2 or 3 divided doses.

If a good therapeutic response has been obtained, treatment can be continued at a dosage adjusted on an individual basis. If no improvement is observed within 10 weeks, treatment with fluvoxamine maleate tablets should be reconsidered. While there are no systematic studies to answer the question of how long to continue Luvox treatment, OCD is a chronic condition and it is reasonable to consider continuation beyond 10 weeks in responding patients. Dosage adjustments should be made carefully on an individual patient basis, to maintain the patient at the lowest effective dose.

The need for treatment should be reassessed periodically. Some clinicians advocate concomitant behavioral psychotherapy for patients who have done well on pharmacotherapy.

Patients suffering from hepatic or renal insufficiency should start on a low dose and be carefully monitored. fluvoxamine maleate tablets should be swallowed with water and without chewing.

Use in pregnancy and lactation

Caution should be exercised when prescribing to pregnant women.

Nevertheless, the usual precautions concerning administration of any drug during pregnancy should be observed. Fluvoxamine is excreted via human milk in small quantities. Therefore, fluvoxamine should not be used by lactating women.

Drug interactions

Fluvoxamine should not be used in combination with MAOIs.

Fluvoxamine is a potent inhibitor of CYP1A2, and to a lesser extent of CYP2C and CYP3A4. Drugs which are largely metabolised via these isoenzymes are eliminated slower and may have higher plasma concentrations when co-administered with fluvoxamine. This is particularly relevant for drugs with a narrow therapeutic index. Patients should be carefully monitored and, if necessary, dose adjustment of these drugs is recommended.

Fluvoxamine has marginal inhibitory effects on CYP2D6 and seems not to affect non-oxidative metabolism or renal excretion.

CYP1A2

An increase in previously stable plasma levels of those tricyclic antidepressants (e.g., clomipramine, imipramine, amitriptyline) and neuroleptics (e.g., clozapine, olanzapine) which are largely metabolised through cytochrome P450 1A2 when given together with fluvoxamine, has been reported. A decrease in the dose of these products should be considered if treatment with fluvoxamine is initiated.

Patients co-administered fluvoxamine and CYP1A2 metabolised drugs with a narrow therapeutic index (such as tacrine, theophylline, methadone, mexiletine) should be carefully monitored and, if necessary, dose adjustment of these drugs is recommended.

When given with fluvoxamine, warfarin plasma concentrations were significantly increased and prothrombin times prolonged.

Isolated cases of cardiac toxicity have been reported when fluvoxamine was combined with thioridazine.

As plasma concentrations of propranolol are increased in combination with fluvoxamine, the propranolol dose may need to be lowered.

Caffeine plasma levels are likely to be increased during co-administration with fluvoxamine. Thus, patients who consume high quantities of caffeine-containing beverages should lower their intake when fluvoxamine is administered and adverse caffeine effects (like tremor, palpitations, nausea, restlessness, insomnia) are observed.

As plasma concentrations of ropinirole may be increased in combination with Fluvoxamine thus increasing the risk of overdose, surveillance and reduction in the posology of ropinirole during fluvoxamine treatment and after its withdrawal may be required.

CYP2C

Patients co-administered fluvoxamine and CYP2C metabolised drugs with a narrow therapeutic index (such as phenytoin) should be carefully monitored and, if necessary, dose adjustment of these drugs is recommended.

CYP3A4

Terfenadine, astemizole, cisapride.

Patients co-administered fluvoxamine and CYP3A4 metabolised drugs with a narrow therapeutic index (such as carbamazepine, ciclosporin) should be carefully monitored and, if necessary, dose adjustment of these drugs is recommended.

The plasma levels of oxidatively metabolised benzodiazepines (e.g. triazolam, midazolam, alprazolam, and diazepam) are likely to be increased when co-administered with fluvoxamine. The dosage of these benzodiazepines should be reduced during co-administration with fluvoxamine.

Glucuronidation

Fluvoxamine does not influence plasma concentrations of digoxin.

Renal excretion

Fluvoxamine does not influence plasma concentrations of atenolol.

Pharmacodynamic interactions

The serotonergic effects of fluvoxamine may be enhanced when used in combination with other serotonergic agents (including triptans, SSRIs and St. John's Wort preparations).

Fluvoxamine has been used in combination with lithium in the treatment of severely ill, drug-resistant patients. However, lithium (and possibly also tryptophan) enhances the serotonergic effects of fluvoxamine. The combination should be used with caution in patients with severe, drug-resistant depression. In patients on oral anticoagulants and fluvoxamine, the risk for haemorrhage may increase and these patients should therefore be closely monitored. As with other psychotropic drugs patients should be advised to avoid alcohol use while taking fluvoxamine.

Overdosage, symptoms and treatment

Symptoms

Symptoms include gastro-intestinal complaints (nausea, vomiting and diarrhoea), somnolence and dizziness. Cardiac events (tachycardia, bradycardia, hypotension), liver function disturbances, convulsions and coma have also been reported.

Fluvoxamine has a wide margin of safety in overdose. Since market introduction, reports of deaths attributed to overdose of fluvoxamine alone have been extremely rare. The highest documented dose of fluvoxamine ingested by a patient is 12 grams. This patient recovered completely. Occasionally, more serious complications were observed in cases of deliberate overdose of fluvoxamine in combination with other drugs.

Treatment

There is no specific antidote to fluvoxamine. In case of overdose the stomach should be emptied as soon as possible after tablet ingestion and symptomatic treatment should be given. The repeated use of medicinal charcoal, if necessary accompanied by an osmotic laxative, is also recommended. Forced diuresis or dialysis are unlikely to be of benefit.

Dosage forms and packaging available

10 tablets x 10 piece (PTP Package), 10 pieces per box.

10 tablets x 3 piece (PTP Package), 3 pieces per box.

Storage conditions

Store at temperature below 30°C. Protect from light and keep out of reach of children.

Shelf-life

24 months from the date of manufacture if kept as recommended.

Registration number

ANWU ENTERIC COATED TABLETS 50mg – MAL09XXXXXXA

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